OET Nursing: Review and Test-Taking Strategies

A complete guide to all four sub-tests, from the NextGen Nurses team

How to use this guide: Read the overview first, then work through one sub-test at a time alongside official OET practice tests. Every section ends with the most common mistakes, so check yourself against them after each practice test. OET and Ahpra update their rules from time to time; always confirm the current format on the official OET website and the current scores on Ahpra's accepted English tests page before you book.

1. The test at a glance

Nurses sit the **Nursing** version of OET. Listening and Reading are the same for all professions; Writing and Speaking use nursing scenarios.

Sub-test	Time	Structure	How it is marked
Listening	About 40 min	Part A: 2 consultations (24 note-completion questions) Part B: 6 short workplace extracts (6 questions) Part C: 2 talks or interviews (12 questions)	42 questions, 1 mark each
Reading	60 min	Part A: 4 short texts, 20 questions (strict 15 min) Part B: 6 workplace extracts (6 questions) Part C: 2 long texts (16 questions)	42 questions, 1 mark each
Writing	45 min	5 min reading time + 40 min to write one letter (e.g. referral, discharge or transfer) from case notes	6 criteria, marked by trained assessors
Speaking	About 20 min	Two role plays (about 5 min each), 3 min preparation for each; the interlocutor plays the patient or carer	9 criteria, recorded and marked by assessors

Delivery modes

OET can be taken on paper at a test venue, on computer at a test venue, or at home with remote supervision (OET@Home). The structure and marking are the same. On computer, the Writing letter is typed. Before you book, check that Ahpra accepts the mode you choose.

2. Scores you need

Each sub-test is reported on a scale of 0 to 500 with a grade from A (highest) to E. Grade B starts at 350.

Grade	Score range	What it means
A	450 to 500	Very fluent and accurate in all healthcare situations
B	350 to 440	Effective in most healthcare situations; the usual registration standard
C+	300 to 340	Can maintain interaction but with noticeable errors
C	200 to 290	Limited effectiveness
D / E	0 to 190	Not yet effective

NMBA minimum scores (tests taken on or after 23 April 2026)

Listening	Reading	Writing	Speaking
350	360	350	360

Tests taken on or before 22 April 2026 use the earlier minimums. In Listening and Reading, Grade B usually needs around 30 or more correct answers out of 42, but the exact cut-off changes from test to test.

3. Listening

Part A: Consultation notes

You listen to two recorded consultations (about 5 minutes each) and complete a health professional's notes. You hear each recording once.

- Use the reading time to scan the headings and gaps. Predict the type of word you need: a symptom, a time, a medication, a number.
- The notes follow the order of the conversation. If you miss an answer, move on and catch the next one.
- Write exactly what you hear. Do not paraphrase or add extra words.
- Spelling counts. Practise common medical words, drug classes and symptoms until you spell them without thinking.
- Listen for corrections: the patient may say one thing, then change it ("Tuesday... no, actually Wednesday").
- Write singular or plural exactly as you hear it (*headache* versus *headaches*).

Part B: Workplace extracts

Six short extracts from healthcare workplaces such as handovers, team briefings and staff conversations. One question each, three options.

- Read the question and context line before the audio starts. Underline who is speaking and what is asked.
- Ask yourself: what is the **main point** or **purpose** of this conversation?
- Distractors often use words you hear in the audio but with a different meaning. Choose the option that matches the meaning, not the words.

Part C: Presentations and interviews

Two longer recordings (about 5 minutes each) with six questions each, three options.

- Many questions test the speaker's **opinion or attitude**. Listen for signals such as *I'd argue, what surprised me, the real issue is, to be honest*.
- The answer often comes after words like *but, however, actually*.
- Questions follow the order of the recording. Keep your place by moving your pen down the page.

Common Listening mistakes

- Spelling errors in otherwise correct answers.
- Writing a whole sentence when a single word or short phrase fits.
- Choosing an option because it repeats words from the recording.
- Losing concentration after missing one answer.

4. Reading

Part A: Expeditious reading (15 minutes, strict)

Four short texts on one topic, such as a medication. You match information, answer short questions and complete sentences. Papers are collected after 15 minutes, so speed is everything.

- **Scan, don't read.** Look at the question first, then find the key word in the texts.
- Learn what each text looks like: a table, a dosage chart, a guideline, a case summary. Some questions are answered faster from the layout than from the words.
- Copy answers exactly as written in the text, with correct spelling. Do not change the word form.
- Never leave a gap: an educated guess costs nothing.
- Practise with a timer every time. Aim to finish in 13 minutes so you have 2 minutes to check.

Timing: about 45 seconds per question.

Part B: Workplace texts

Six short texts such as policies, emails, notices and guidelines, with one three-option question each.

- Read the question first, then the text. Most questions ask about the purpose of the text or what staff must do.
- Watch for small but important words: *must, should, may, unless, only, all*.
- Eliminate options that are true but don't answer the question.

Timing: about 2 to 3 minutes per text.

Part C: Long texts

Two articles of about 800 words on healthcare topics, with eight four-option questions each. These test detail, opinion and vocabulary in context.

- Skim the whole text first (2 minutes) to understand the overall argument.
- Questions follow the order of the text. Locate the paragraph, then read it carefully.
- For "what does *it* or *this* refer to" questions, read the sentence before.
- For vocabulary questions, replace the word with each option and see which keeps the meaning.
- Separate the writer's opinion from the opinions of people the writer quotes.

Timing: about 20 minutes per text, leaving 5 minutes to check.

Common Reading mistakes

- Reading Part A in full instead of scanning.
- Spending too long on one Part C question. Mark it, move on and come back.
- Picking an option based on general knowledge rather than the text.

5. Writing

You receive case notes and a task, usually to write a referral, discharge, transfer or advice letter. You have 5 minutes to read (no writing) and 40 minutes to write. The suggested length is 180 to 200 words for the body of the letter.

The six marking criteria

Criterion	What the assessor checks
Purpose	Is the reason for writing clear in the first paragraph, and expanded later?
Content	Is all the information the reader needs included, and is it accurate?
Conciseness and clarity	Is irrelevant detail left out and is the letter easy to follow?
Genre and style	Is the tone professional and suitable for the reader (a doctor, a community nurse, a carer)?
Organisation and layout	Are paragraphs logical and is the letter format correct?
Language	Are grammar, vocabulary, spelling and punctuation accurate?

The 5-minute reading time

- Read the **task** first: who is the reader, why are you writing, and what do they need to do?
- Read the case notes with the reader in mind. Mentally mark each note as *needed*, *maybe* or *not needed*.
- Decide your paragraph order before the writing time starts.

Selecting the case notes

Not every note belongs in your letter. Ask: "Does this reader need this to care for the patient?"

- **Include:** the reason for referral, the current condition, relevant history, current medications, social factors that affect care, and the actions you are asking for.
- **Leave out:** normal findings that don't affect care, minor history, and detail the reader already knows.
- Group related notes together instead of following the order of the case notes.
- Put the most urgent information first.

Letter structure

Use this outline as a guide. The details in square brackets come from your case notes.

```
Ms Jane Smith
Community Nurse
[Community Health Centre]
[Address]

[Date]

Dear Ms Smith,

Re: Mr John Brown, aged 72

Para 1 PURPOSE: I am writing to refer Mr Brown, who is being
discharged today following [procedure / admission], for
[specific care needed].
Para 2 Admission and current condition.
Para 3 Relevant history, medications, social situation.
Para 4 REQUESTS: what you need the reader to do, and when.

Please do not hesitate to contact me if you require any further information.

Yours sincerely,

Nurse
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Writing tips

- State the purpose in your first sentence. The assessor should know why you are writing within 10 seconds.
- Write in full sentences and paragraphs. Do not use bullet points or note form.
- Turn notes into sentences: *"BP 150/90, SOB on exertion"* becomes *"His blood pressure was 150/90 and he reported shortness of breath on exertion."*
- Use the correct tense: past for the admission, present for the current condition, future for the plan.
- Avoid abbreviations the reader may not know. Write *twice daily* rather than *BD* if the reader is not a clinician.
- If the case notes give the recipient's name and address, include them. Use *Yours sincerely* when you use the reader's name and *Yours faithfully* with *Dear Sir/Madam*.
- Sign as "Nurse" or with the name and title given in the task.

Timing: 5 minutes reading, about 5 minutes planning, 30 minutes writing, 5 minutes checking.

Useful phrases for letters

Purpose	Useful phrases
Opening the purpose	I am writing to refer... / I am writing regarding... who is being discharged today...
Background	Mr X was admitted on [date] with... / On admission, she presented with...
Current status	She is currently... / His condition has improved and he is now able to...
History	Her medical history includes... / He has a history of...
Social context	She lives alone and... / He is supported by his daughter, who...
Requests	I would be grateful if you could... / Could you please monitor...
Closing	Please do not hesitate to contact me if you require any further information.

Common Writing mistakes

- Copying every case note, which makes the letter long and unclear.
- Hiding the purpose in the second or third paragraph.
- Using note form, bullet points or informal language.
- Forgetting to say what the reader should do next.
- Tense and article errors (*a, an, the*), which affect the Language score.

6. Speaking

You complete two role plays. In each one you are the nurse and the interlocutor plays a patient, relative or carer. You have 3 minutes to read the role card and prepare, and about 5 minutes for the conversation. A short warm-up at the start is not marked.

The nine marking criteria

Group	Criterion	What it means
Linguistic	Intelligibility	You are easy to understand: clear pronunciation, stress and rhythm
	Fluency	You speak at a natural pace without long hesitations
	Appropriateness of language	Your words suit a patient: no jargon, polite and professional
	Resources of grammar and expression	You use a range of accurate grammar and vocabulary
Clinical communication	Relationship building	You introduce yourself, show empathy and respect
	Understanding the patient's perspective	You ask about and respond to their ideas, concerns and expectations
	Providing structure	You signpost and move through the conversation logically
	Information gathering	You use open questions, listen and summarise
	Information giving	You explain in small chunks and check understanding

Using the 3 minutes of preparation

- Identify your setting, who the patient is and the **tasks** listed on the card. You must cover every task.
- Plan an opening line and a simple order for the tasks.
- Think of plain-English explanations for any medical terms on the card.
- Predict the patient's likely concern or resistance; the card often hints at it.

A reliable role-play structure

- **Open:** greet, introduce yourself, confirm the patient's name and explain the purpose.
- **Explore:** ask open questions about how they feel and what worries them, and listen.
- **Explain:** give information in small chunks in plain language.
- **Check:** ask the patient to say the plan back in their own words (teach-back).
- **Plan and close:** summarise, agree next steps, ask if they have questions, and thank them.

Useful phrases for role plays

Purpose	Useful phrases
Opening	Hello, I'm [name], one of the nurses on the ward. Could you confirm your full name for me?
Showing empathy	That sounds really difficult. / I can understand why you'd feel worried about that.
Patient's perspective	What are your main concerns about...? / What do you already know about...?
Signposting	First, I'd like to ask about... then I'll explain... / Now, let's talk about...
Open questions	Can you tell me more about...? / How has this been affecting you at home?
Summarising	So, if I've understood correctly, you've been feeling...
Checking understanding	Just so I know I've explained it clearly, could you tell me how you'll take it?
Handling resistance	I understand your concern. Can I explain why this is important?
Closing	Is there anything else you'd like to ask? Thank you for talking with me today.

Common Speaking mistakes

- Talking too much: a monologue loses marks for relationship building and information gathering.
- Using medical jargon such as *hypertension* without a plain-English explanation.
- Ignoring the patient's worry and moving straight to information.
- Running out of time before covering every task on the card.
- Reading phrases from memory in a flat voice. Sound natural and warm.

7. A 6-week study plan

Week	Focus
1	Take a full practice test to find your weakest sub-test. Learn the format of every part.
2	Listening Part A and Reading Part A daily with a timer. Build a medical spelling list.
3	Writing: one letter every two days. Practise selecting case notes in under 5 minutes.
4	Speaking: two role plays a day, recorded. Listen back and check against the nine criteria.
5	Listening and Reading Parts B and C. Review every wrong answer and why the distractor fooled you.
6	Two full timed practice tests. Rest the day before the exam.

8. Test-day checklist

- Bring the same ID you used to book. The name must match exactly.
- Arrive early (or set up early for OET@Home) to complete check-in calmly.
- In Listening, use every second of reading time to scan the questions.
- In Reading Part A, watch the clock. Papers are collected at 15 minutes.
- In Writing, spend the first 5 minutes planning in your head, then write and leave time to check.
- In Speaking, smile, slow down and treat the interlocutor like a real patient.

> Practice is the key. Work through official OET practice tests, record yourself in Speaking, and have your letters checked against the six criteria. Good luck from the **NextGen Nurses** team!

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